

Infant and Pediatric Abduction (Code Pink)

Policy Number	POL-EM-057	Owner / Department	Security / Women's & Children's Services
Effective Date	2023-11-15	Version	2.0
Last Revised	2026-03-06	Next Review	2028-03-06
Approved By	Emergency Management Committee	Applies To	All staff, with primary roles in Women's & Children's Services, Pediatrics, the Nursery/NICU, and Security

1. Purpose

To prevent the abduction of an infant or child and to mount an immediate, coordinated recovery response if one occurs, consistent with National Center for Missing & Exploited Children (NCMEC) guidelines, The Joint Commission Emergency Management and security standards, and the facility infant security (electronic tagging) program.

2. Scope

Applies to all areas serving infants and children, especially the Nursery, NICU, Labor & Delivery, Postpartum, and Pediatrics. Every staff member participates in a Code Pink response: monitoring exits, stairwells, and elevators, and observing anyone carrying an infant or a bag/package that could conceal one.

3. Definitions

- **Code Pink** — the overhead alert for a missing or abducted infant; **Code Purple** (or Code Pink–Pediatric, per facility convention) is used for a child.
- **Infant security system** — electronic tags that alarm and may lock doors/elevators if a tagged infant approaches or passes an exit.
- **Abductor profile (NCMEC)** — typically a female visitor who may impersonate staff, becomes familiar with routines, and carries a large bag; most abducted infants are recovered, and planning assumes rapid response saves the child.

4. Procedure

IMPORTANT — A suspected infant abduction is a SEV-1 sentinel event. The moment an infant is unaccounted for, announce Code Pink, notify Security and the Administrator-on-Call, and call 911 / law enforcement immediately — activate HICS. Staff lock down and monitor all exits, stairwells, and elevators, and discreetly observe anyone leaving with an infant or a large bag. Do not wait to confirm before calling.

1. Confirm the infant is missing and announce Code Pink with the unit; notify Security and the charge nurse immediately.
2. Call 911 / law enforcement immediately and notify the Administrator-on-Call; activate HICS.

3. Initiate lockdown: staff proceed to and monitor all exits, stairwells, and elevators; preserve the infant's room as a scene.
4. Gather and broadcast a description of the infant and of any suspect/vehicle; observe and delay (do not physically confront) anyone matching the profile.
5. Account for all infants and children on the affected units; provide the parents a private area and support.
6. File an event report and preserve all evidence (footage, visitor logs, tag data) for law enforcement and Risk Management.

5. Severity & Reporting

An infant or pediatric abduction is a **SEV-1 sentinel event** under POL-RM-013. Notify the Administrator-on-Call, Security, and law enforcement **immediately** (within minutes, never later than 1 hour) and activate HICS. Risk Management coordinates mandatory sentinel-event management under POL-RM-004, including a comprehensive systematic analysis (RCA²) within 45 business days. An infant or child merely missing from the unit (not abducted) is handled as an elopement under POL-SEC-010 and is still at least **SEV-2**, escalating to SEV-1 if the child cannot be located or is harmed.

6. Documentation Requirements

- Infant/child identity, last-seen time and location, and who discovered the absence.
- Description of any suspect and vehicle; whether the infant security tag alarmed.
- Exits/elevators secured, lockdown time, and staff assignments.
- Law-enforcement notification and response; HICS activation and all notifications.
- Evidence preserved (video, visitor logs, tag data) and chain of custody.

7. References

- NCMEC, For Healthcare Professionals: Guidelines on Prevention of and Response to Infant Abductions.
- The Joint Commission, Emergency Management (EM) and security / environment-of-care standards; infant-abduction sentinel-event alerts.
- CMS Emergency Preparedness Rule, 42 CFR 482.15.
- Related: POL-RM-013 (severity & reporting), POL-RM-004 (sentinel event / RCA²), POL-SEC-010 (elopement / missing patient), POL-EM-053 (HICS).